

• SLIDE 01 · HOOK

Being allowed in is not the same as being understood.

You may be in the same room. You may have access to a doctor.
You still may not have access to healthcare.

THE MOMENT

A patient walks in sick and scared. The clinician speaks medical English. The patient's language is Yoruba, Pidgin, Swahili, or Luganda. CareBridge closes that gap — in real time.

2

languages in the room

0

shared clinical words

Language and health literacy block care — even when care is present.

Inclusion is not putting a hospital where people can reach it. It is making the healthcare experience understandable once they get there.



Second-language intake

Patients cannot accurately describe symptoms, history, or pain in a language that is not theirs.



Opaque instructions

A patient may “read” English and still not understand a prescription, discharge note, or consent form.



Downstream risk

Miscommunication drives misdiagnosis risk, wasted clinic time, poor adherence, and loss of trust.

WITHOUT A SHARED LAYER

- ✓ Symptoms filtered through a second language
- ✓ Clinician note ≠ patient's own words
- ✓ Instructions leave the room unclear
- ✓ Trust and follow-up erode



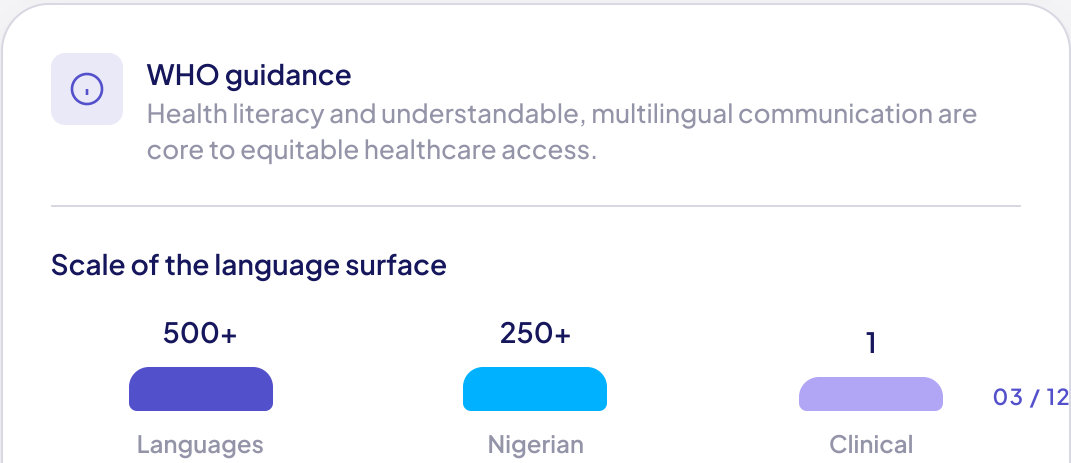
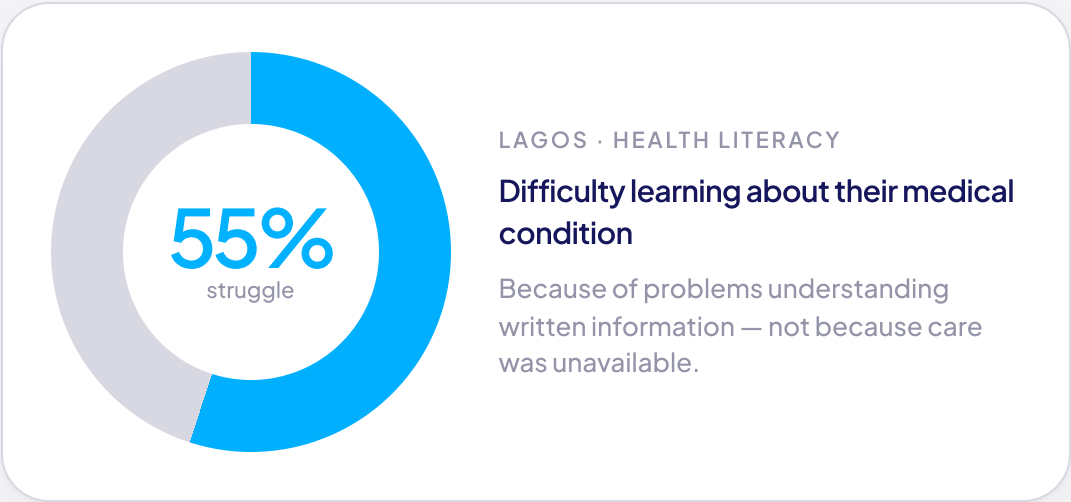
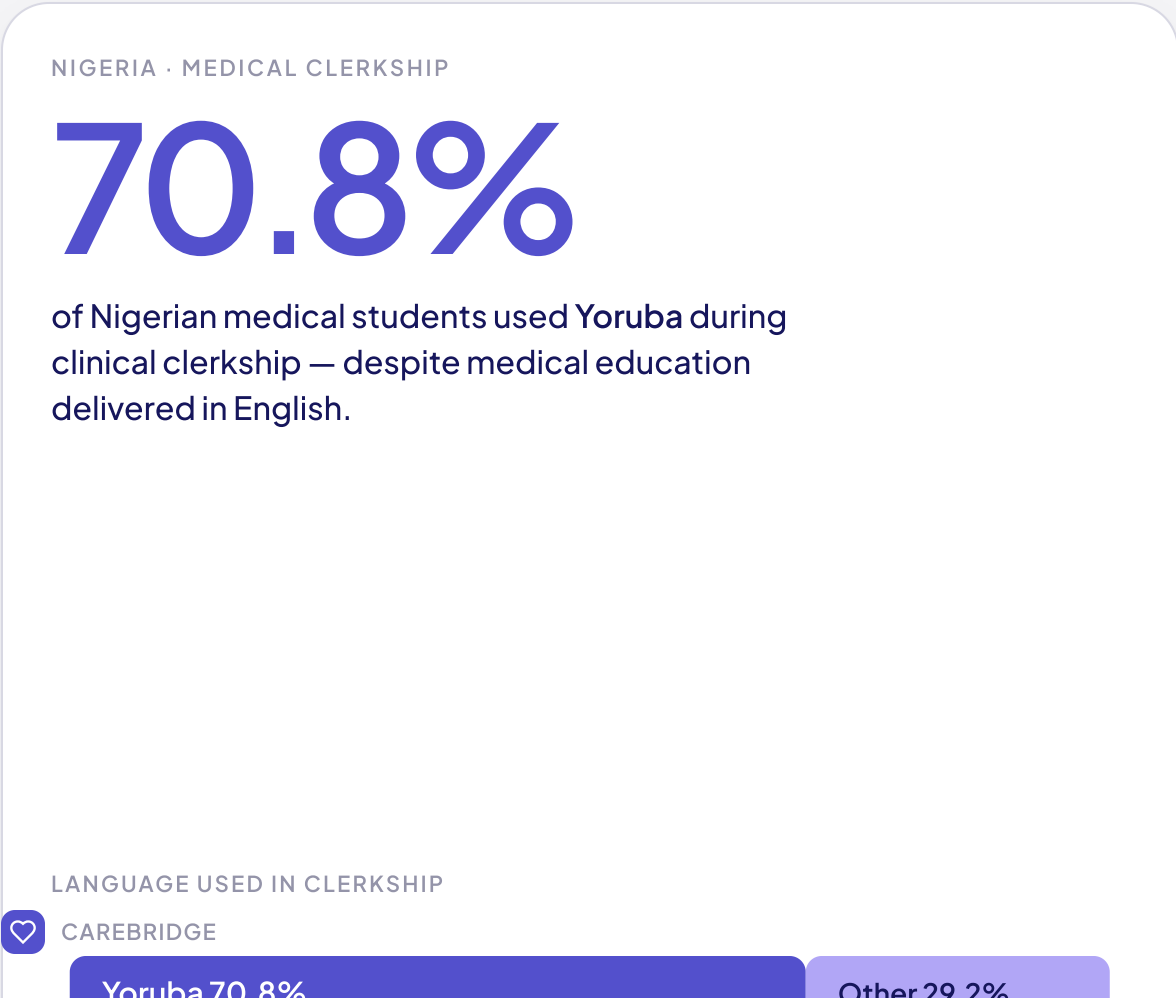
WITH CAREBRIDGE

- ✓ Patient speaks naturally in their language
- ✓ Structured clinical summary for the clinician
- ✓ Response returned as simple text + speech
- ✓ Both sides leave aligned



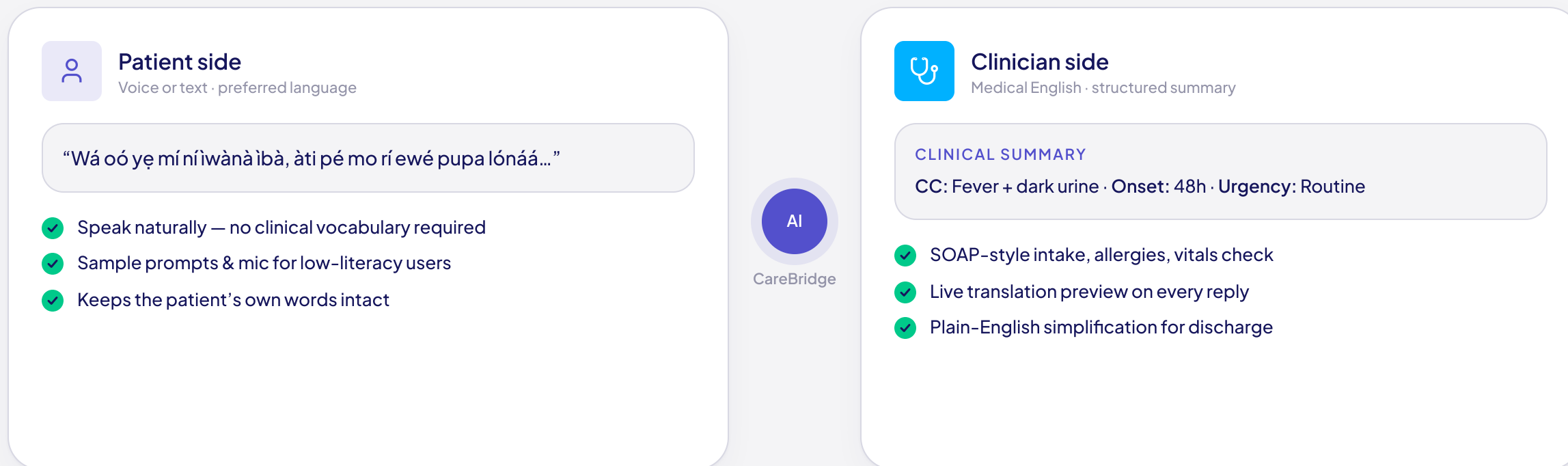
The gap is documented — not anecdotal.

Research from Nigerian clinical training and Lagos health-literacy studies, aligned with WHO guidance on equitable communication.



An AI communication layer between patient and clinician.

One conversation. Two languages. Zero information lost — across language, literacy, and accessibility barriers, in the same room.



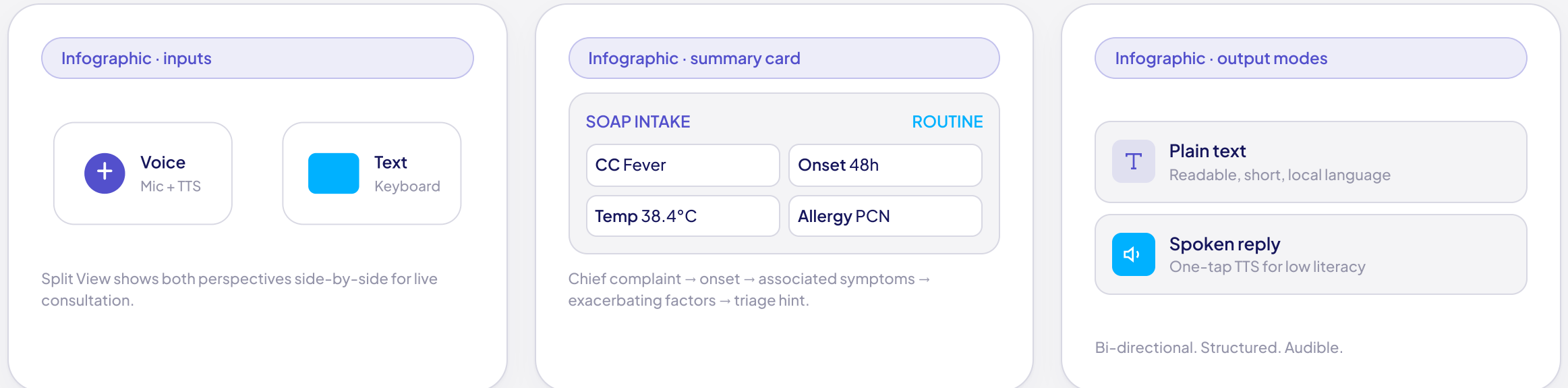
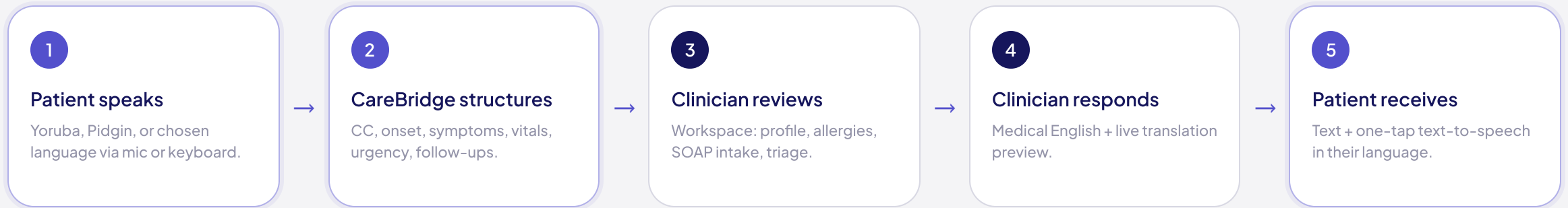
Loop closes

Clinician responds in professional language → CareBridge returns simple language as **text or speech** in the patient’s language.



A structured, two-way translation loop.

Not raw machine translation — clinical structuring on the way in, plain-language simplification on the way out.



What CareBridge is — and is not.

A hard line around scope, on purpose. Communication infrastructure only — clinical judgment stays with the healthcare worker.



Not replacing doctors

CareBridge never sits between clinician judgment and the patient as a substitute provider.



Not diagnosing patients

No diagnostic claims. Structured summaries inform the clinician; they do not replace examination.



Not a consumer paywall

We do not ask sick people to pay for access to their own care conversation.

We solve this

Making sure the patient can be **heard** — and can **understand** the person responsible for their care.

Be heard in your language

Leave with clear instructions

Clinician sees structured facts

Accessibility built in

Clinical judgment stays with the healthcare worker.



Built for real clinics — a working PWA, not slide-ware.

Voice, text, and plain-language communication with accessibility at the core. Installable, offline-ready, demo-script guided.

LANGUAGES · V1

gb English

ng Nigerian Pidgin

ng Yoruba

Voice · Text · Speech out



Installable PWA

Home screen, no app store. Service worker + precache for low bandwidth.



Accessibility first

Text size, high contrast, skip link, reduced motion, spoken replies.



60s demo script

Guided happy path for presentations: voice → summary → reply → speech.

PATIENT VIEW

Voice-first intake, sample prompts, listening playback, live thread.

CLINICIAN VIEW

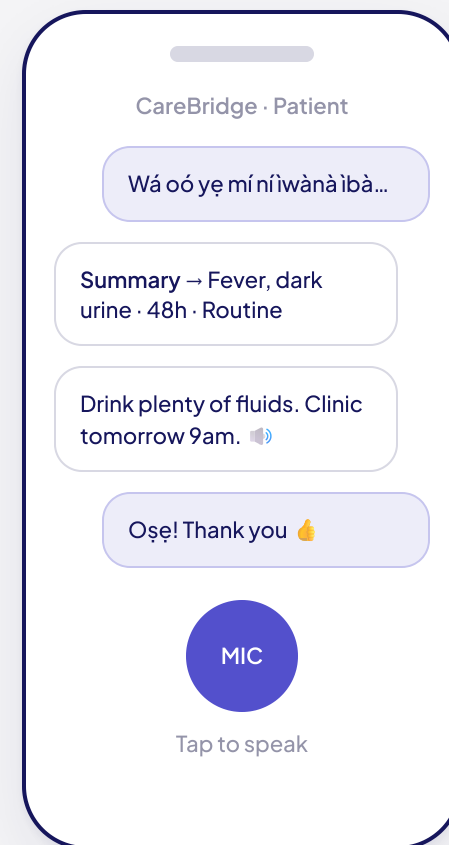
Profile, vitals, structured summary, jargon simplifier, live preview.

SPLIT VIEW

Both sides on one screen for real-time consultation demos.

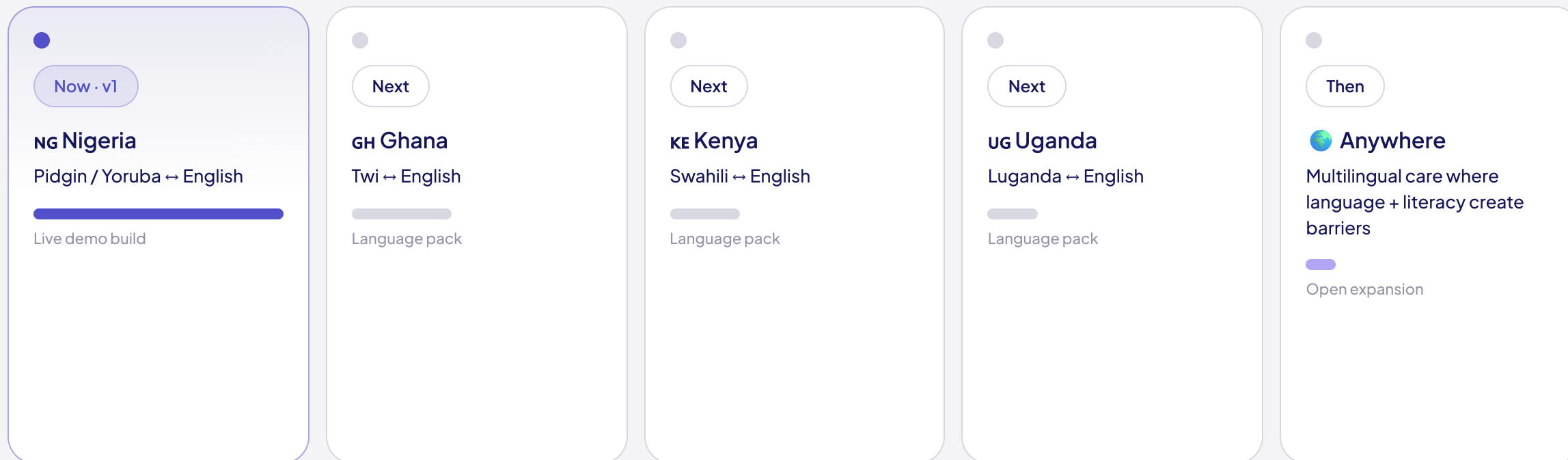
Stack

Bun · TypeScript · Vite · Tailwind · PWA · type-safe build



Language-agnostic architecture. Continent-first roadmap.

Same product, new language packs. We do not rebuild to enter a market — we extend.



WEDGE

Start where pain is sharpest — multilingual Nigerian clinics.

GROW BY

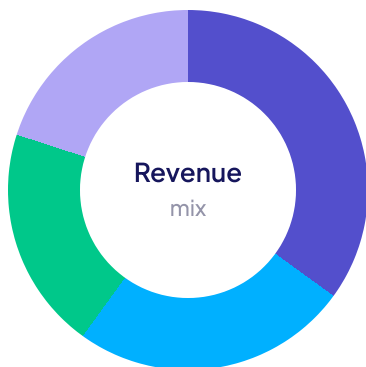
Language packs, not feature sprawl.

UNIT OF SCALE

Clinic → country → continent → global health systems.

Providers pay. Patients benefit.

Not a consumer app that bills sick people. Institutions that provide access become the customers.



- Clinic subscriptions
- Enterprise deployments
- NGO / public health
- Platform API

Illustrative mix for pilot-stage planning.



Clinics & hospitals

Subscription per facility or per healthcare worker

Core



Large health systems

Enterprise deployment + admin controls

Scale



NGOs & public health

Underserved communities · mission-aligned pricing

Impact



Health platforms

API integration into existing products

API

B2B

Principle: The people who need better access receive the benefit. The institutions providing that access become the customers.



Timing and wedge are aligned.

Distribution, model quality, and policy pressure have converged — and we already have a working end-to-end build.



PWA reach

Smartphone + installable web app works in low-resource clinics — no app-store friction.



Reach without install gates



AI quality threshold

Speech + translation are good enough for structured clinical summarization and plain-language output.

VOICE → SUMMARY → REPLY → TTS

End-to-end loop demonstrated live.



Policy tailwind

WHO and ministries actively push health literacy and equitable multilingual communication.

Equitable access = priority



Hackathon-proven

Working demo, installable PWA, a11y audited, type-safe Bun + TypeScript + Vite.

PWA ✓

A11y ✓

tsc ✓

3 languages

Wedge

Start where the pain is sharpest — multilingual Nigerian clinics — then expand **language-by-language**, not feature-by-feature.



Communication infrastructure for equitable healthcare.

Help systems serve more people — without requiring every healthcare worker to speak every language.

Long-term vision

Language should never be the reason care fails.

CareBridge becomes the layer every clinic, CHW, and EMR relies on when patient and provider do not share a language.

UG Uganda

CHW triages in Luganda → English summary for referral hospital.

Field

Referral

NG Nigeria

Discharge instructions in Pidgin audio — not just English paper.

Audio

Adherence

EMR

Store the patient's *own words* alongside the clinician's note.

Record

Continuity



Access should mean being **understood** when you get there.

Be heard. Be understood. Help us pilot CareBridge where language barriers meet the clinic door.



Pilot clinics

1–2 clinics, live Yoruba / Pidgin ↔ English workflow.



Partners

NGO / public-health program serving multilingual communities.



Feedback loops

Clinician + patient usability sessions to harden triage quality.



Build partners

Speech / translation contributors: Twi, Swahili, Luganda packs.

CareBridge

be heard. be understood.

PWA · Live demo ready

Partner with us →

